



P 20 · 40 PAGES, FOR ANY WEEK

A Gentle Companion for Pregnancy Loss

Forty pages for anyone who has lost a pregnancy at any week. Permission-slip pages. A "what to say to people" script library. Physical recovery notes (by class, never by name). Commemorative pages. A page for the partner. A page for the next pregnancy, whenever that comes. Not a workbook. A companion.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

Before you open this

If you are in crisis right now, please put this companion down for a moment and call one of these.

- **988**, US Suicide & Crisis Lifeline (call or text, 24/7)
- **1-833-852-6262**, US Maternal Mental Health Hotline (24/7, free, confidential, for any parent at any stage including loss)
- **741741**, Crisis Text Line (text "HOME" to start)

If you have heavy bleeding, severe pain, signs of infection, or anything physically worrying, please go to your provider or to an emergency room. Loss has medical aspects. Those are for a real provider, not for this companion.

If you are not in crisis but you are in the long stretch that comes after, this companion is for that stretch. Take it slowly. Skip pages. Read the same one twice. Cry on it if you want. The pages can hold it.

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A note from me

I am not a therapist. I am not a doctor. I am someone who has been in a quiet room with a wand-and-screen and watched the technician's face go still. I am someone who has called their partner from the parking lot. I am someone who has had a friend lose at twenty-four weeks, and another at six, and a third in the second trimester of a much-wanted pregnancy after years of trying.

What those friends and I learned, over hours and years, is that pregnancy loss does not come with a script. The world goes on. People say wrong things. Your body does things you weren't ready for. The "supposed to" you had been carrying for weeks or months sets down somewhere, and you cannot pick it back up.

This companion is the thing I wish I'd had. Not to fix anything. Not to "heal" anyone on a timeline. Just to keep a record. To give some sentences to the difficult conversations. To not be alone in the quiet rooms.

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What this companion is, and is not

IS

- A book to hold during the long, quiet weeks.
- Permission slips for things you did not know you were allowed to want.
- Script libraries for the conversations you do not have the words for.
- A space for your own writing, drawing, or silence.
- A page for the partner.
- A page for whenever, in the future, the loss surfaces again.

IS NOT

- A clinical guide. We do not name medications. We do not give medical instructions. We route every medical question to your provider.
- A timeline. There is no “stage of grief” map in here. Grief does not follow a map.
- A religion or a spirituality. If you have one, it can layer over this. If you don’t, the companion does not require one.
- A “you’ll heal” promise. We will not tell you what you will feel in three months.
- A roadmap to a next pregnancy. That conversation belongs to you and a real provider, on your own timeline, if at all.

What loss looks like, in plain words

Pregnancy loss happens in different shapes. Each one carries its own grief. They are not in any rank order. Every one is real loss.

Early loss (weeks 4-13)

Often called "miscarriage." Sometimes the loss is detected before symptoms (an ultrasound finds no heartbeat). Sometimes it begins with bleeding or cramping. Either way, the body is doing a thing that may or may not require a procedure.

This is the most common, statistically (about one in four recognized pregnancies). It is also the form most often dismissed by other people, with phrases like "at least it was early." It was your loss. Early or not, it was yours.

Missed loss

The pregnancy stops developing, but the body does not yet know. This often comes at a scan, where the screen is quiet and the technician does not narrate.

The grief here often includes the strangeness of being told, in plain words, that the thing you have been carrying is no longer alive. The body has not yet caught up. You may have to make decisions about how the body will catch up. None of those decisions are simple.

Ectopic pregnancy

The pregnancy is growing in the wrong place (often a fallopian tube). This is medically urgent. You may have had emergency care, and that care may have included the loss of an organ. There is often physical recovery, surgical recovery, and emotional recovery all at once.

This companion is here for the third one. The first two are for your provider.

Stillbirth (week 20 or later)

A pregnancy lost in the second half. This is medically and emotionally a different stretch. The body has labored. You may have held the baby. You may have named them.

Most of this companion's later pages are written with you in mind. If you are reading this just after, please go gently with yourself. There is no schedule.

Termination for medical reasons (TFMR)

A loss that came with a decision. The pregnancy carried a diagnosis incompatible with life, or with carrying the pregnancy to term safely. You made a choice. You are also grieving.

This is one of the loneliest forms of loss because it does not fit the cultural script. The companion holds it. We do not have an opinion about the choice. We have only the love and the loss.

Donor / IVF / surrogacy losses

The grief is the same. The financial weight, the planning, the wait, can make the loss feel especially heavy. Use this companion for the loss part. The other parts are real too, and they deserve their own conversations.

Permission slips

Six pages, one permission each. You can copy them, photograph them, hand them to someone, or just hold them.

Permission 1

Today, I do not have to be okay.

I do not have to tell anyone.

I do not have to make sense of anything.

I do not have to be productive.

I can sit on the floor.

I can not eat dinner.

I can not return calls.

The world will be there tomorrow.

So will the people who love me.

Permission 2

I am not in a hurry to "feel better."

I am not behind. There is no schedule.
There is no week in which this is supposed
to be smaller.

My body and my mind are doing their own
work, at their own pace. I do not have to
speed them up.

Permission 3

I am allowed to skip the baby shower.

I am allowed to mute the pregnancy
announcement.

I am allowed to leave a holiday early.
I am allowed to not RSVP.

I do not have to explain. I am taking
care of the person I am right now.

Permission 4

I am allowed to grieve a future I had
already started imagining.

The room I had pictured.
The name I had not said out loud.
The version of the next year that
is no longer.

None of those are "silly." They were
real. They are real losses.

Permission 5

I am allowed to cry where I want to.

In the car. In the bathroom at work.
In the cereal aisle. In bed at 3am.

I am also allowed to NOT cry, and have
that not mean anything is wrong with me.

Grief does not always look like tears.

Permission 6

I am allowed to not have a "reason" for what happened.

Most losses do not have a reason. The ones that do, the reason is often medical and unspecific, and does not answer the question.

The question, often, was "why me." That question does not have an answer. The companion will not pretend it does.

A “what to say to people” script library

The most exhausting part, often, is the people who do not know what to say and try anyway. Here are scripts for the most common moments.

When a text comes in

You do not have to respond the same day. You do not have to respond at all.

If you want a short response, ready to send:

"Thank you for thinking of me. I'm not really talking about it yet. I'll reach out when I am ready."

If you want to say more:

"Thanks for the message. It's been a lot. I am taking it slowly. I will tell you more when I can."

When someone says “at least”

(As in: “at least it was early.” “At least you can get pregnant.” “At least you have one already.”)

You can:

"This is hard. The 'at least' makes it harder. Can we just be sad about it for a minute?"

Or, the version where you do not have the energy to teach them:

"Mm. Thank you."

(Then change the subject. You do not owe an education.)

When someone says “everything happens for a reason”

"I am not at the point where I can hear that.
I might never be. I appreciate that you mean well."

When a relative keeps asking when you’re trying again

"That's not a question I'm able to answer right now. I will tell you if we have news."

(Then change the subject. Repeat as needed, for months.)

When a pregnant friend does not know what to say

"I am happy for you. I am also sad. Both are true. You don't have to do anything about my sad. Just keep being my friend, the way you always have."

(If a pregnant friend pulls away because they think their joy is hurting you, sometimes they need permission to stay close.)

When your boss asks

If you do not want to share details:

"I had a medical event. I am okay. I'd appreciate not discussing it further, if that's all right."

If you do want to share:

"I lost a pregnancy. I am going to need [time off / a slower pace / flexibility on ____]. I appreciate the support."

When a stranger asks "is this your first?" (in a future pregnancy)

This question, in a later pregnancy after loss, lands like a hit. A response that is true and does not require you to explain:

"I have one baby. I am working on the next."

Or, if you want to honor the loss out loud:

"This is my second baby. The first did not get to be born."

Or, simply:

"It's complicated. Thank you for asking."

Physical recovery, in plain language

A page. We will not name medications. We will not give procedures. We will name what's typically happening, and route every specific to your provider.

What is typical

After a loss, the body returns to a non-pregnant state over a stretch of days to several weeks. The hormones drop. The uterus contracts. Bleeding (lochia) is common, similar in many ways to postpartum bleeding, sometimes lighter, sometimes heavier, depending on the loss.

Cramping is typical for days to weeks.

A first cycle usually returns within 4-8 weeks for early losses, longer for later losses. Your cycle may behave differently for several months. That is also typical.

What to call about

- Bleeding heavy enough to soak a pad in less than an hour
- Severe pain that does not lift with rest
- Fever
- Foul-smelling discharge
- Pain on one side that is sharp or growing
- Any sign you suspect of infection

These are not edge cases. They warrant a call. Today.

What your provider can help with

- Pain that is more than the typical post-loss range
- Sleep that is not coming back

- A milk-supply situation. If a late-term loss, your body may produce milk. Your provider can help.
- The first cycle if it does not return within 8 weeks
- The conversation about whether and when to try again, if you want one

What this companion will not say

We will not name any medication. We will not name any procedure. We will not estimate how you will feel in a number of weeks. The body's recovery is yours and your provider's. The mind's is yours.

A page for the pregnancy that did not finish

A page with very few words on it. You can write, draw, leave blank, paste a small thing, or skip.

What I will hold for you:

A second page, the same:

If I could tell you one thing:

A third page, with a small square for a date or a name if you have one or both:

A date that matters: _____

A name (if there is one): _____

What I want to remember:

Some readers will fill all three. Some will fill none. Some will fill them years apart. All of those are correct.

A page for the partner

For the partner who did not carry the pregnancy. You are often handed the role of “be strong.” This page says: you do not have to be.

To the partner:

You are also grieving.

You may have been told (or you may have told yourself) that your job is to be the steady one. The one who handles the calls and the casseroles. The one who takes care of her.

You can be that, sometimes. Not all the time.

Your grief is real. The baby you were already loving is also lost to you. The future you were rearranging your life for is also gone from yours.

You are not "less" sad. You are sad differently, and the difference does not make it smaller.

Things that are okay:

- ☐ Crying alone in the car
- ☐ Crying with your partner
- ☐ Not crying at all and wondering whether that means something is wrong with you (it doesn't)
- ☐ Asking your own friend to take you out and not talk about it for an hour
- ☐ Telling your partner what you need, even when you feel you should not be the one who needs anything
- ☐ Calling the mental-health hotline, the numbers are at the back of this companion

You are her teammate. You are also a person. Both can be true.

A page for the people around you

A page to print, hand out, text, or tape to a wall. So that people who love you know how to actually help.

To our friends and family,

We lost a pregnancy. Thank you for caring about us.

Things we have found helpful:

- ☐ A text that says "thinking of you, no need to reply"
- ☐ A meal left on the porch
- ☐ A coffee dropped off
- ☐ A walk together with no agenda
- ☐ Someone asking, after weeks, "how are you, really" (we forget that we are still in this)
- ☐ Saying the baby's name (if there was one)
- ☐ Acknowledging it on anniversaries, even briefly

Things that have not helped, even though they were kind:

- ☐ "Everything happens for a reason"
- ☐ "At least it was early"
- ☐ "At least you can get pregnant"
- ☐ "You can always try again"
- ☐ "It wasn't meant to be"
- ☐ Asking when we are trying again
- ☐ Pregnancy announcements that pretend we are not in the room

Thank you for being here. We are still here.
We are doing the slow work.

With love,

_____ + _____

A page for the next, whenever it comes

For some readers, there will not be a “next.” This page is not an assumption. It is just here, in case you want it, sometime.

When you are ready (or when you are pregnant again, on whatever timeline that is), here are some things to know.

1. The first appointment will be emotionally loud. You will sit in the same waiting room. The walls will not have changed.

Bring someone if you can. Bring a small anchor in your pocket (a stone, a coin, something you can press).

2. The question "is this your first?" will land hard. You can choose your own answer. Write it down ahead of time so you don't have to think of it in the room.

Some options:

- ☐ "It's complicated, thank you."
- ☐ "I have one I'm carrying now."
- ☐ "This is my second. The first did not get to be born."

3. Tell your provider about the previous loss. Even if they have your chart. Say it out loud. The provider will adjust accordingly (often more frequent check-ins, more compassion).
4. The first ultrasound after a loss can be hard. Tell the technician you have had a loss. They will often narrate as they go, instead of going quiet.
5. You may not feel "happy" about the pregnancy for a long time. That does not mean you do not love it. It means your

body is protecting itself.

6. There is no week at which the previous loss stops mattering. There is also no week at which the new pregnancy stops being its own thing. Both are true.
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An anniversary page

A page for the dates that find you. Some you will mark. Some will surprise you.

A date that matters: _____

Something I might do, when this day comes around:

- ☐ Light a candle
- ☐ Write a sentence on this page
- ☐ Take a walk somewhere quiet
- ☐ Tell my partner the date
- ☐ Have a cup of tea, alone, on purpose
- ☐ Nothing, on purpose
- ☐ _____

The first year:

The second year:

The third year:

And on.

You will not always know how you will mark the day. You may not mark it at all some years.
Both are honest.

What grief does, over time

A page with no specific reading order. The truths that other parents have shared, the year after.

- The acute weeks are real. They are also not the whole of grief. After the acute weeks, grief becomes a thing that lives in the room.
- It does not “go away.” It moves in. It takes a chair. It gets quieter, often, but it does not leave.
- You will be surprised by the moments it speaks up. A baby item in a friend’s house. A song. A waiting room. The week the baby would have been a year old. The first day of school the baby would have started.
- You will also be surprised by the moments it stays quiet. Whole weeks will pass. You will feel guilty for not thinking about the loss. The guilt is not a thing you owe. The forgetting is not a betrayal.
- Some relationships will deepen because of this. Some will quietly thin out. Both will happen. Both are part of what grief does.
- Many parents who have been through loss become a soft place for other parents going through it. You do not have to take that role. You can. If you do, your version of softness will be specific. There is no template.

A page for the version of you reading this in two years

A page to close the companion.

You are reading this two years (or five, or ten) after the loss. The version of you reading this is not the same as the one who picked the companion up.

The loss did not become smaller. You grew a little around it. That is what people mean by “carrying” something.

Look back at the pages you filled in. Read the names, the dates, the sentences you wrote when you were not sure what to write.

None of that has changed. The pregnancy that did not finish is still real. The version of you who was carrying them is still real. The future you imagined and grieved is still real.

And you are also here, today, in your body, with whoever is with you, with whatever shape your family now is.

Both are true. Both stay true.

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Numbers to keep

PROVIDER (OB / midwife / GP)

Name: _____ Number: _____

THERAPIST (general or grief-specialized)

Name: _____ Number: _____

US MATERNAL MENTAL HEALTH HOTLINE

1-833-852-6262 (24/7, free, confidential, parent-specific)

US SUICIDE & CRISIS LIFELINE

988 (call or text, 24/7)

US CRISIS TEXT LINE

Text "HOME" to 741741

POSTPARTUM SUPPORT INTERNATIONAL (loss + perinatal mood)

1-800-944-4773 (call), text "Help" to 800-944-4773

SHARE, Pregnancy & Infant Loss Support

1-800-821-6819

NATIONAL ALLIANCE ON MENTAL ILLNESS (NAMI) HELPLINE

1-800-950-6264 (Mon-Fri)

A LOCAL GRIEF GROUP

A SAFE PERSON TO CALL



From Soothemade Notes, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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